

NOTE DATED: 04/16/2018 10:54

LOCAL TITLE: MED ONCO IRCH DISCHARGE SUMMARY

STANDARD TITLE: DISCHARGE SUMMARY

VISIT: 04/16/2018 10:53 DR OFFICE

Dr BR AMBEDKAR INSTITUTE ROTARY CANCER HOSPITAL, AIIMS, NEW DELHI-110029
DEPARTMENT OF MEDICAL ONCOLOGY

DISCHARGE SUMMARY

"UNLESS OTHERWISE SPECIFIED ALL DATES MENTIONED IN THIS NOTE ARE IN THE FORMAT MM/DD/YYYY"

REGISTRATION DETAILS

IRCH No: CR No:
AGE:63 GENDER:FEMALE
DOA:Apr 15, 2018 DOD:Apr 16, 2018 DURATION OF STAY:1 days
WARD: MRO Ward BED No:14
CONSULTANT INCHARGE:Dr Sameer Bakshi

DIAGNOSIS & REASON FOR CURRENT ADMISSION

DIAGNOSIS: Ca Breast

ADMITTED FOR :Chemotherapy

CASE SUMMARY: PATIENT WAS ADMITTED FOR 1# Epirubicin + Cyclophosphamide

Inj Epirubicin 110 mg/250 ml NS/1 hr

Inj Endoxan 750 mg/500 ml 5% D/2 hr

Patient received chemotherapy on 15/4/18 with no immediate toxicity

FOLLOW UP

Come for follow up in BCC CLINIC with Biochemistry Report, Hemogram Report .

Signed by: /es/ VIRAJ NEVREKAR
SR MED ONCO
04/16/2018 10:57

Dr. B.R. Ambedkar Institute Rotary Cancer Hospital
ALL INDIA INSTITUTE OF MEDICAL SCIENCES

Height.
Weight
BSA

Name of the patient

Flowsheet Medical Oncology

PATIENT DATE

Date : 4/1/18 9/4/18 26/4/18

Temp Heart Rate Resp Date B.P. Resp. Sys. CVS CNS						
TREATMENT Drugs 1. 2. 3. 4. 5. 6. 7.						
LABORATORY HB/PCV Platelates (K 1000) WBC (X 1000) ANC ESR Blood Sugar Blood Urea/Creatinine Calcium/Phosphate Uric Acid (mg %) Sodium / Potassium Billirubin (mg %) D/I Total Protein (%) A/G Ratio SGOT/SGPT ALK. Phos (Units) Tumour Markers Others	14.2 252x11 ³ 5160 8200	11.8 229x11 ³ 4720 1850 24/0.6 8.8/4.9 0.2 7.5/4.4 21/18 187	12.5 228x11 ³ 3570 2450 2570.6 8.2/5.2 3.2 140/3.5 6.5/3.0 16/20 203 0.4			
REACTIONS TO THEARAPY Nausea Diarrhea Mucositis Fever Others						
SUPPORT DATA BT / PRP I/O						



Department Of Pathology
All India Institute Of Medical Sciences
Delhi

Tel: +91-11-26588500/26588700; Fax: +91-11-26588500/26588700

Patient Name: [REDACTED]

F/H Name: [REDACTED]

Age/Sex: 63 Y/Female

Clinic/Dept/Unit: Dr. B.R. A. IRCH/Unit 1

Reg Date: 10-01-2018

Acc. No: [REDACTED]

Hosp. Reg. No.: [REDACTED]

UHID No.: [REDACTED]

Consultant Incharge: Dr. S.V.S. Deo

Reporting Date: 25-01-2018

Histopathology Report

Report Findings:

Received six specimens

I. Specimen labelled as "left chest wall nodule" measures 2.5x2x1.5cm with overlying skin measuring 2.5x2cm. Cut section showed a grey white tumor measuring 2.3x1.8x0.6cm.

-Sections examined show features of an invasive carcinoma, NST.

-The hormonal profile of the tumor is as follows:

ER-positive(8/8)(Allred score) ✓

PR-positive(8/8)

Her 2 neu-3+; The ki67 proliferative index is 12-15%

The following specimens sent separately are free of tumor

II. Medial margin

III. Lateral margin

IV. Inferior margin

V. Deep margin

VI. Superior margin

Impression: Recurrent carcinoma breast.

Reporting Incharge: Dr. Shipra Agarwal

Reporting SR: Dr. Nishu

Verify By: Dr. Hemlata

ECHOCARDIOGRAPHY REPORT

DEPARTMENT OF CARDIOLOGY, CARDIOTHORACIC CENTRE
ALL INDIA INSTITUTE OF MEDICAL SCIENCES, NEW DELHI-110029

NAME..... AGE 63/4 SEX M/F DATE 10/4/18
ECHO No. CV No. UHID No. C.R. No.
HEIGHT.....cm WEIGHT.....kg. BSA.....m² Ref. Physician J.R.-C.1
Referring Diagnosis
Quality of Imaging Poor/Adequate/Good Done by Dr. Sushant Checked by Dr.

MITRAL VALVE

Morphology AML - Normal/Thickening/Calcification/Flutter/Vegetation/ Prolapse/ SAM/ Doming
PML Normal/ Thickening / Calcification/ Prolapse/Paradoxical motion/Fixed.
Subvalvular deformity Present/ Absent Score.....
Doppler Normal / Abnormal
Mitral stenosis Present / Absent RR interval.....msec
EDG.....mmHg MDG.....mmHg MVA.....cm²
Mitral regurgitation Absent/Trivial/Mild/Moderate/Severe

TRICUSPID VALVE

Morphology Normal/ Atresia/ Thickening/ Calcification/ Prolaps/ Vegetation/ Doming
Doppler Normal/ Abnormal
Tricuspid stenosis Present/Absent RR interval.....msec
EDG mmHg MDG.....mmHg
Tricuspid regurgitation Absent/Trivial/Mild/Moderate/Severe Fragmented Signals
Valocity.....m/sec Pred. RSVP-RAP+.....mmHg

PULMONARY VALVE

Morphology Normal/Atresia/Thickening/Domng/Vegetation
Doppler Normal/Abnormal
Pulmonary stenosis Present/Absent Level
PSG.....mmHg Pulmonary annulus.....mm
Pulmonary regulation Present/Absent
Early diastolic gradient.....mmHg End diastolic gradient.....mmHg

AORTIC VALVE

Morphology Normal / Thickening/Calcification/Restricted Opening/Flutter/Vegetation No. of cusps 1/2/3/4
Doppler Normal / Abnormal
Aortic stenosis Present/Absent Level
PSG.....mm Hg Aortic annulus.....mm
Aortic regurgitation Absent/Trivial/Mild/Moderate/Severe

Measurements

Aorta
LV es
IVS ed
RV ed
EF
IVS Motion
IAS

20
26
11
55-60

Normal Values

(21-22mm/m²)
(16-19mm/m²)
(06-10mm)
(4-14mm/m²)
(62-80%)

LA es
LV ed
PW(LV)ed
RV Anterior wall

28
30
11

Normal Values

(21-22 mm/m²)
(19-32 mm/m²)
(07-11mm)
(upto 5mm)

CHAMBERS

LV
LA
RA
RV

Normal/Enlarged/Clear/Thrombus/Hypertrophy
Contraction Normal/Reduced
Normal/Enlarged/Clear/Thrombus
Normal/Enlarged/Clear/Thrombus
Normal/Enlarged/Clear/Thrombus

PERICARDIUM

Normal/Thickened/Calcification/Effusion.

REMARKS

Normal chambers

No clot

TEE

No RMA

Normal AV function EF 55-60%

DIAGNOSIS

Final Impression

normal study

Resident

Consultant

Department of Nuclear Medicine and PET-CT
All India Institute of Medical Sciences, New Delhi, India.



Patient Name: [REDACTED]

Age/Sex : 60Y/F

Study ID: [REDACTED]

Date: 31.01.2018

Indication: C/o Carcinoma left breast

^{99m}Tc - MDP WHOLE BODY BONE SCAN

Findings:

- *Mild linear radiotracer uptake seen in shaft of bilateral femora and an area of mild radiotracer uptake in shaft of left tibia- ?stress related changes ?reactive*
- Normal radiotracer distribution noted in rest of the skeleton.
- Bilateral kidneys are visualized.

Impression:

- No definite scan evidence of skeletal metastasis.

Note: Bone scan has low sensitivity for purely lytic lesions.


Senior Resident

Consultant

RADIOLOGY UNIT

Dr. B. R. Ambedkar Institute Rotary Cancer Hospital
All India Institute of Medical Sciences, New Delhi-110029

Patient Name: [REDACTED]	Age/Gender: 63 Years and 9 Months / F
Patient UHID: [REDACTED]	IRCH No: [REDACTED]
Accession No: [REDACTED]	Location: SO OPD
Date of Examination: 28-FEB-2018 10:19 AM	

Procedure: CECT of Chest, Abdomen & Pelvis.

Clinical background: Carcinoma left breast – post MRM. Now C/O small nodule over scar site.

Chest

Both lungs: Normal. There is a 5mm sized nodule seen along oblique fissure. Likely benign

Mediastinum: Normal. No lymph node enlargement.

Trachea, main bronchi: Normal.

Serosal spaces: No pleural or pericardial effusion.

Left breast tissue not seen – post op. No obvious mass lesion seen in the operative bed. There is a 8mm sized subcentimetric left axillary node seen.

Abdomen & Pelvis

Liver: Normal. No focal lesion.

CBD/ Gall bladder: Normal.

Pancreas: Normal.

Spleen: Normal.

Both kidneys/ ureters: Normal. No hydronephrosis.

Retroperitoneum/ vessels: Normal. No lymph node enlargement.

Free fluid: No ascites.

Urinary bladder: Normal.

Uterus/ adnexa: Normal.

Additional information: None.

Comparison: None.

Impression

Post op case of Carcinoma breast with no obvious mass lesion in operative bed and no metastases.

Radiologist: Dr. Keshav Gupta (SR) / Dr. Mukesh Kumar



**All India Institute of Medical Sciences
Department of Pathology**

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Histopathology Report

Name: [REDACTED]	Acc.No [REDACTED]
C/O : [REDACTED]	Received on : 2014-05-28
Age : 60 years	Hosp.Reg.no. : [REDACTED]
Sex: F	Reporting Date : 2014-06-10
DR. B.R.A. IRCH Consultant Incharge:	

Material:

Report:

Received 3 specimens.

I. Left mastectomy specimen measuring 12x12x8cm, overlying skin measuring 11x5cm. Grossly overlying skin, nipple and areola are unremarkable. On cut section, a solid grey white tumor measuring 2x1x1cm is identified in the central quadrant. The tumor lies 0.8cm from deep resected margin and 1cm from overlying skin.

-Multiple sections examined from the tumor show features of invasive duct carcinoma, NOS, Grade -II.

(Nottingham's modification of Bloom Richardson score).

Rest of the breast parenchyma shows fibrocystic change.

-Overlying skin, nipple and areola are free of tumor.

II. Specimen sent separately as ' deep margin' is free of tumor.

III. Specimen sent as ' axillary lymphnode dissection ' measuring 5x4x3.5cm. On dissection, 9 lymphnodes were identified varying in size from 0.3 to 0.8cm in maximum dimension,shows reactive lymphoid hyperplasia, free of tumor (0/9).

(Note: For Hormonal profile refer to previous histopath access no . 14-4208).

Impression: Invasive duct carcinoma NOS, Grade II, left breast.

TNM staging - P (T1c N0 Mx) within limitation of metastatic workup.

Dispatch Date: 10-06-2014

Typed By: Sunita

Senior Resident:

Dr. Tarun Kumar

Consultant Histopathologist: Dr. Geetika Singh

Discharge Summary, Surgical Oncology, IRCH, AIIMS

Name	[REDACTED]	Age	60	Sex	F
DOA	19-May-2014	DOS	28-May-2014	DOD	31-May-2014
IRCH No.	[REDACTED]	SOFU No.	517/2014		
Address	[REDACTED]				

Diagnosis Carcinoma left breast

Stage rT2N1M0

**Comorbid
Conditions** None

Clinical Presentation Post lumpectomy recurrent lump left breast for 5 months

General Examination ECOG 1

Local Examination Scar present at 12 o clock left breast, induration present. 3x3 cm lump present left central quadrant.

**Preoperative
Treatment** Lumpectomy 14/1/14 Pvt

Surgical Procedure LEFT MRM

Operative findings Finding : Scar present superior aspect of NAC with 4x4 cm palpable lump. Skin, NAC, chest wall normal. Subcentemetric LN present at axilla level I.
Procedure : Left MRM done. Pectoralis minor, LD pedicle, LTN preserved. Axillary clearance done upto level II.

Postoperative Course uneventful

Treatment Plan As per HPR

Advise at Discharge Tab Flexon Bd for 5 days
tab Rantac BD for 5 days

To Come date 06-Jun-2014 **Followup Place** 9b @ 1:30 pm

**Haematological
Details**

Tumor Markers

Chest X Ray NAD

Mammogram 1/2/14 Right breast normal
Left breast BIRADS 2 post op changes

Skeletal Survey

Bone Scan

USG Details abdomen 4/2/14 Normal study

CT Details

MRI Details

Endoscopy Details

Tissue Diagnosis 144208 IDC/ER+/PR+/Her2+

HPE Report Awaited

Senior Resident

Dr Ashish

Consultant

Prof SVS Deo

SURGICAL ONCOLOGY, IRCH, AIIMS
Operative Procedure, Findings, Treatment Plan

Name	[REDACTED]	Age	60	Sex	F
IRCH No.:	[REDACTED]	DOS	28-May-2014		
		SOFU No.	[REDACTED]		

Diagnosis Carcinoma left breast

Stage rT2N1M0

Preoperative Treatment Lumpectomy 14/1/14 Pvt

Surgical Procedure LEFT MRM

Operative findings Finding : Scar present superior aspect of NAC with 4x4 cm palpable lump. Skin, NAC, chest wall normal.
Subcentemetric LN present at axiila level I.
Procedure : Left MRM done. Pectoralis minor, LD pedicle, LTN preserved. Axillary clearance done upto level II.

HPE Report Awaited

Treatment Plan As per HPR

Senior Resident

Ashish
Dr Ashish

Consultant

Prof SVS Deo

**DR. B.R.A. INSTITUTE ROTARY CANCER HOSPITAL
ALL INDIA INSTITUTE OF MEDICAL SCIENCES**

NEW DELHI-110029

DEPARTMENT OF ANAESTHESIOLOGY

NAME [REDACTED] AGE / SEX 60/F C.R. NO. [REDACTED] DATE 6.2.14

DIAGNOSIS (STAGE) G1 Breast PROPOSED OPN. Completion mastectomy

CLINICAL HISTORY (BRIEF)

CVS - Breathlessness/Palpitation/Chest Pain ☒ HT/CAD ☒

RS - Cough/Hemoptysis ☒ Asthma ☒

CNS - Seizure/Headache/TIA/CVA/Neurological Deficit ☒ TB ☒

Abdomen - Distension/Vomiting/Hepatitis/Renal Disease ☒

Endocrine - DM/Thyroid Disease/Parathyroid Disease ☒

H/O Allergy Not known

Any other significant History Food cooking by chullah 40 years.

CURRENT / PAST MEDICATION

1. Anti HT ☒
2. Anti Diabetic ☒
3. Anti Thyroid ☒
4. Bronchodilators ☒
5. Steroids ☒
6. Chemotherapy Drugs ☒
7. Radiotherapy Received ☒
8. Any Other Drugs ☒

PAST ANAESTHETIC HISTORY

Lumpectomy done 4 GA 1m ago -> u/s at Almada (UK)

Weight <u>40 Kg</u> Height Pulse <u>78/min</u> BP <u>92/64 mmHg</u> R.R. BHT	General Physical Examination - Temperature - Pallor - Icterus - Cyanosis ☒ - Clubbing - Oedema - Venous Access ☒	CVS <u>S/S + Mo</u> Resp. System <u>B/L A/C</u> Abdomen / CNS <u>NAD</u>
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Airway Assessment :

Mouth Opening 3F
 Loose Teeth/Buck Teeth/Dentures/Edentulous/Missing Teeth Loose teeth + ulcers incisor
 Mallampati Score III
 Neck Examination : Movements adequate
 Radiation Induced Changes
 DIFFICULT AIRWAY ANTICIPATED
 Spine :

Broken teeth & chipped
poor dentition & Hygiene

Receding Mandible ☒ Subluxation ☒
 Thyromental Distance 3F Mentohyoid Distance
 Post Surgical Deformity
 YES/NO ☒

8:15

9:15

10:15

TIME		100	100	100	100	100	100	100																
SPO2/E.CO.		36	38	40	38	37	37	38																
C	260																							
40	240																							
38	220																							
36	200																							
34	180																							
	160																							
28	140																							
24	120	✓	✓	✓	✓	✓	✓	✓																
20	100	✓	✓	✓	✓	✓	✓	✓																
16	80	✓	✓	✓	✓	✓	✓	✓																
12	60	○	○	○	○	○	○	○																
08	40	△	△	△	△	△	△	△																
04	20																							
TEMP.																								
CVP																								
D INHAL		Sevo 0.6		0.5		0.6																		
R NARC		F100																						
U RELAX		R30																						
G OTHERS		P100																						
S		by Perfalgan 1gm		by Dynapars 75mg		by Esmect-Pmg																		
IVF RA		(P&G) → NS ₁		→ NS ₂																				
LA																								
URINE OUTPUT		Not catheter used																						
EVENTS																								

20/5/14
 Hb 11.1
 TLC 4300
 Tpr 7.0
 ALB 4.0
 GUB 3.0
 INVESTIGATIONS (Date) Set 0.7
 2

Hb/Hct	WBC/PLATELETS	SUGAR F/PP/R	UREA/CREATININE	NA/K/CA
13.1	4500/284000	92.7	83/0.7	141/4.4
T PROTEINS / A : G	BILIRUBIN / D / I	SGOT / PT	ALKP04ASE	HBsAG / HIV
7.4/4.1/3.3	1.0	36/32	142	

X RAYS (Date) CXR - NAD No emphysematous changes			ECG/ECHO/MUGA (Date) ECG - NAD WNL	
PFT	Actual	% Predicted	USG/CT/MRI	
FVC			OTHERS RBS → 113 mg/dl 27/5/14	
FEV ₁				
FEV ₁ /FVC				
PEFR				

RISK GROUP STATUS (ASA). _____ Reason for risk _____

FURTHER ORDERS AND INVESTIGATIONS

✓ DATE FOR SURGERY CAN BE GIVEN

* NEEDS FURTHER INVESTIGATIONS

ADMIT _____ DAYS PRIOR TO SURGERY

AND INFORM SR. ANAESTHESIOLOGY

SEEN BY (CAPITALS)

SIGNATURE

DESIGNATION

DATE

Review PAC (Date)

PRE-OP MEDICATION (DRUG) DOSE ROUTE TIME

NPO after

Consent : Routine / High Risk

3. Sedative

4. Narcotic

5. Antisialogogue

6.

7.

8.

9.

10

Investigations to be done on Morning of Surgery

Tab Rantac 150mg HS & QM
 Tab alprax 0.25mg HS & QM

Jan

PAC order

27/5/14
 No JIC
 Chart CUS (N)
 P- 80/Min
 BP- 100/60

adv → Get RBS charting done
 → B.S Morning of Sx
 → Get SF done on Morn
 → NPO - Midnight
 → follow pre PAC order

SR

ORDERED BY

SIGNATURE

NAME

DATE/ TIME

ANAESTHESIA RECORD

ANAESTHESIOLOGISTS: Dr. Sushma, Dr. Vidhu

SURGEONS: Dr. Shukla, Dr. Ashish, Dr. Mahender

SURGERY PERFORMED: ② MRM

TECHNIQUES: GA/REGIONAL/COMBINED

NAME: [REDACTED] AGE/SEX: 60/F CR NO: [REDACTED] DATE: 28/5/14 VENTILATOR: Auye

POSITION: Supine IV ACCESS RT: 18G LT:

TIDAL VOL /MV: 325ml

CVP: Art:

PAWP: 11cm H₂OGA: Induction: O₂ + f100 + P100 + Roc30

FREQUENCY: 12/min

↳ Igel no - ③

CIRCUIT: ①

INTUBATION - ORAL/NASAL /LARYNGSCOPIC/FIBER OPTIC/OTHERS

LAR YNGO GRADE: INTUBATION EASY / DIFF / IMPOSS

NO. OF ATTEMPTS ①

CALCULATED BLOOD VOL.

MAINTENANCE: O₂ + Au + servo

CALCULATED FLUID REQ

REGIONAL ANAESTHESIA:

1st hr 4th hr

SAB/ EPID / CSEA/NERVE BLOCK

2nd hr 5th hr

SITE: NEEDLE GAUGE:

3rd hr 6th hr

DRUG CONC: VOLUME:

CALC BLOOD LOSS:

ANTIBIOTICS: iv Augmentin 1.2gm

BLOOD REPLACED:

OTHER DRUGS: iv Dynapar 75mg, iv Emetet 8mg

BLOOD GROUP:

iv Paralogan 1gm

BLOOD BAG NO.:

REVERSAL: iv Neo 2.5 GIVEN AT 9:45am

EXTUBATION: PGlyco 0.4mg

T - PIECE

ELECTIVE VENTILATION: RK

BEFORE SHIFTING TO ICU:

CONSCIOUSNESS: PULSE 80/min BP 124/80 RESP 16/min SPO₂ 100% CVP:**EVENTS / COMPLICATIONS:****FLUID BALANCE:**

1. ESTIMATED BLOOD LOSS: minimal
2. BLOOD REPLACED: nil
3. COLLOIDS REPLACED: nil
4. CRYSTALLOIDS INFUSED: 600ml
5. URINE OUTPUT: nil

Vidhu
Signature of Anaesthetist

DR. B.R. AMBEDKAR INSTITUTE ROTARY CANCER HOSPITAL
ALL INDIA INSTITUTE OF MEDICAL SCIENCES

UNIT OF ANESTHESIOLOGY
ANESTHESIA SUMMARY

SURGERY PERFORMED : (L) MRM

NAME [REDACTED]

AGE/SEX 60/F

IRCH NO. [REDACTED]

DATE 28/5/14

ANESTHESIOLOGISTS : Dr. Sushma, Dr. Vidhu

SURGEONS : Dr. Deo, Dr. Ashish, Dr. Mahender

PAC FINDINGS (RELEVANT) : (I)

WT-40kg
PR-78/min
BP-120/86 mmHg

ASA GRADE :

Igel no - (3)

LARYNGO GRADE :

INTUBATION : EASY/ DIFF./ IMPOSSIBLE / BLIND / BRONCHOSCOPIC

INDUCTION	MAINTENANCE	TUBE / AIRWAYS	SIZE
1. INTRAVENOUS (Thio/prop/Ket) 2. INHALATIONAL (Iso/Hal) 3. RAPID SEQUENCE 4. OTHERS O ₂ + F ₁₀₀ + P ₁₀₀ + Roc30	1. INHALATIONAL 2. TOTAL I.V. 3. BALANCED 4. OTHERS O ₂ + Au + Sev	0. FACE MASK 1. OROTRACHEAL 2. NASOTRACHEAL 3. TRACHEOSTOMY 4. DOUBLE LUMEN 5. LARYNGEAL MASK Igel no (3) 6. OTHERS	TUBE TYPE 0. NONCUFFED 1. CUFFED 2. RED RUBBER 3. PVC 4. RAE

BREATHING SYSTEM	VENTILATOR TYPE	REGIONAL	LEVEL OA
MAGILL BAIN CLOSED - PARTIAL CLOSED - TOTAL OTHERS	Aesys	1. SAB 2. EPIDURAL 3. BRACHIAL 4. BIRS 5. OTHERS	CATHETER

FLUID BALANCE :

ESTIMATED BLOOD LOSS :

minimal

BLOOD REPLACED :

nil

COLLOIDS REPLACED :

nil

CRYSTALLOIDS INFUSED :

600ml

EVENTS / COMPLICATIONS :

afe

INTRAOPERATIVE :

POST OPERATIVE :

shifted to RR

28/5/14
@ 1200 PM

Pt con, oriented
P-73

BP 100/70

SpO₂ - 98% on room air

UAS < 3

adv by Refalgan 1gm IV T.D.S

- by dynapae 75mg IV B.D.

Shift to ward

~~SR~~
SR

Name:	[REDACTED]	IRCH No:	[REDACTED]
Age/sex:	60 F	OPD/Ward	OPD
Investigation:	Ultrasound	Date:	04/02/2014

Report

Procedure: Ultrasound of Abdomen & Pelvis.

Clinical Background: Carcinoma left breast.

Liver: normal. No focal lesion is seen.

Common Bile Duct/intrahepatic bile ducts: Not dilated.

Gall bladder: Normal.

Pancreas: Normal.

Spleen: Normal.

Kidneys/ ureters: Normal. No hydronephrosis.

Retroperitoneum/ vessels: Normal. No lymph node enlargement.

Free fluid: No ascites or pleural effusion.

Urinary bladder: normal.

Uterus & adnexa: normal.

Additional information: None.

Impression: Normal scan.

Comparison: None.

Senior Resident

[Signature]
SR



Cert. No. M-0137

CLIENT CODE : [REDACTED]

CLIENT'S NAME AND ADDRESS :

POOJA DIAGNOSTIC CENTRE
SHOP NO. 7 & 8, NEAR BASE HOSPITAL, LOWER MALL ROAD,
KHATYARE
ALMORA 263601
UTTARANCHAL INDIA
59-62236323 9411585380

SRL LIMITED.

SRL REGIONAL REFERENCE LAB, GP-26, MARUTI INDUSTRIAL
ESTATE, UDYOG VIHAR, SECTOR-18,
GURGAON, 122015
HARYANA, INDIA
Tel : 1800-102-8282/1800-222-660, Fax : 0124-4591001
Email : srlrri@srlcnet.com

PATIENT NAME : [REDACTED]

PATIENT ID :

ACCESSION NO : [REDACTED]

AGE : 42 Years

SEX : Female

DATE OF BIRTH :

DRAWN : 08/01/2014 05:10

RECEIVED : 10/01/2014 12:38

REPORTED : 14/01/2014 15:34

REFERRING DOCTOR : DR. VINITA JOSHI

CLIENT PATIENT ID :

Test Report Status **Final**

Results ✓

HISTOPATHOLOGY**MEDIUM BIOPSY SPECIMEN,
TISSUE/PARAFFIN BLOCK***** HISTOPATHOLOGY REPORT**

METHOD : MICROSCOPY

ADDITIONAL COMMUNICATION

10-01-2014, TISSUE KEPT FOR FIXATION.

METHOD : MICROSCOPY

SPECIMEN

BREAST LUMP.

METHOD : MICROSCOPY

GROSS

CLINICAL HISTORY SHEET MENTIONS SPECIMEN AS BREAST LUMP,
RECEIVED IN FORMALIN ARE MULTIPLE GREY BROWN SOFT TISSUE
PIECES MEASURING 5 x 4 x 2 CMS IN AGGREGATE.
REPRESENTATIVE SECTIONS ARE SUBMITTED IN 4 BLOCKS.

*** INTERPRETATION****DIAGNOSIS**

✓ INVASIVE DUCTAL CARCINOMA, NOS, BREAST.
NOTTINGHAM HISTOLOGIC SCORE
TUBULE FORMATION - 3.
NUCLEAR PLEOMORPHISM - 2.
MITOSIS - 3.
TOTAL SCORE - 8, GRADE 3. ✓
LYMPHOVASCULAR EMBOLI NOT SEEN.

COMMENTS

COMMENT ON STATUS OF MARGINS IS NOT POSSIBLE AS SPECIMEN WAS RECEIVED IN MULTIPLE PARTS.

Comments

The comments printed in 'Request letter' / 'Additional Communication' are defined actions undertaken by the laboratory prior to release of final report.

****End Of Report****

Please visit www.srlworld.com for related Test Information for this accession
TEST MARKED WITH '*' ARE OUTSIDE THE NABL ACCREDITED SCOPE OF THE LABORATORY.

Dr. Anu Chawla
Pathologist

Dr. Priti Jain
Sr. Consultant Histopathology

Bhagwan Mahavir Hospital, Pitampura, Delhi, 110034 Govt. Of NCT of Delhi OPD TICKET			
Deptt. : Surgery, ,	Room No. : 108, ,	UID :	OPD Regn No : 31531
Name [REDACTED]	Age : 60 Y/ 0 Mnt/0 Day	Sex: Female	Date : 30/1/2014 Time : 9 : 38 : 20 AM
Location/Area :			Referral to Deptt Room No.
Provisional Diagnosis			ICD-10 Code

K/c/o - Ductal carcinoma. (L)
breast.

① Lumpectomy has done 14/01/2014.

O/E → Palpable lump at
excision site.
No chest wall involvement
No palpable axillary lymph
node.

- Referred to department
of oncology, AIIMS, New Delhi.

OPD NO. :31531
Referral to Deptt
ICD-10 Code :

OPD NO. :31531
Referral to Deptt
ICD-10 Code :

Dr. PRALAY KUMAR
SILVERGATE
Bhagwan Mahavir Hospital
Govt. of NCT of Delhi
Pitampura, Delhi-110034

[Signature]
30/1/14



**All India Institute of Medical Sciences
Department of Pathology**

Tel: +91-11-26588500/26588700; Fax: +91-11-26588663/26588900

Histopathology Report

Name: [REDACTED]	Acc.No [REDACTED]
C/O [REDACTED]	Received on :2014-02-04
Age : 60 years	Hosp.Reg.no. : [REDACTED]
Sex: F	Reporting Date :2014-02-13
DR. B.R.A. IRCH Consultant Incharge: Prof. N.K. Shukla	

Material: Biopsy, sample excision /1 H&E

Report:

Received one HE stained slide and one block for review as excision biopsy from left breast lump.

- Section cut from the block and corresponding stained slide both show features of an infiltrating duct carcinoma, Nos, grade 2, score 7. Nottingham's modification of Bloom Richardson's score.

Hormone profiles of the tumor are as follow.

Estrogen Receptor - Positive (3 ,90%) = 8/8 (ALLRED SCORE)

Progesterone Receptor - Positive (3 ,90%) = 8/8 (ALLRED SCORE)

Her2/neu receptor - Positive (3).

Dispatch Date: 13-02-2014
Typed By: Pankaj Nagia
Verified By: Dr. Manish Singh

Senior Resident:
Consultant
Histopathologist:

Dr. Debojit
Dr. Asit Ranjan
Mridha

Name: [REDACTED]
Age/sex: 60 F
Investigation: Mammography

IRCH No: [REDACTED]
OPD/Ward: OPD
Date: 01/02/2014

REPORT

Procedure: Bilateral Mammogram.

Clinical Background: Carcinoma left breast (History of lumpectomy in private)

Right Breast:

Parenchyma: Normal. No mass or abnormal calcification is seen. No architectural distortion. Subcutaneous tissues: Normal.
Areola: Normal.
Axilla: No suspicious LNs.

Left Breast:

Parenchyma: Nipple is retracted with surrounding architectural distortion in the deep central quadrant (post op change). No mass seen.
Subcutaneous tissues: Normal.
Areola: Normal.
Axilla: No suspicious LNs.

small intramammary nodule (benign) also seen in infero-medial quadrant of @ breast.

Impression:

Right breast is normal.

BIRADS category-1: Negative (normal) study.

Left breast: Post op change. BIRADS: 2 *(benign post op change)*

Comparison: None

[Signature]
Senior Resident

[Signature]
Consultant

Dr. B.R.A. Institute Rotary Cancer Hospital, AIIMS, New Delhi
MEDICAL ONCOLOGY

C/S by BRSO

30/1/14

C/O lump @ Breast x ~~4~~ 6M. $\approx$ 5x5cm.

↓
Operated for Excising lump 15 days back.

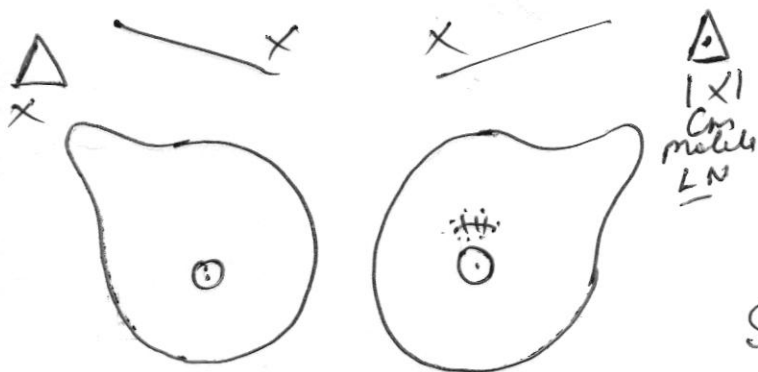
W/S Bx 1x1x1 $\rightarrow$ (U) Breast shows 4.6x 2.7 cm hyper-
Pvt sclerotic lump.

HPE Pvt -
14/1/14

IDC NOS
Grade 3.

O/E. ECOG-0

No Co-morbidities



One Scar mark @
at 12'o clock in (U) Breas
Just Superior to Nipple
areolar complex. C

S. Purulent discharge

- Surrounding skin indurated

C PDD

~~Ble Axilla & Ble LN.~~

(U) Axilla - one mobile 1x1 cm.
LN @

Ca Breast (U)

clinical T₂ N₁ M₀

C/O & Shuckle BIV

adv Plan complete Mastectomy (MAM) @

- R 18
- Slide Review

Flu & Etopos
July

6/2/14

B/L Mammography -

(R) Breast - w/ study

(L) " - Breasts 2 - postop clamps

Doc for Summary:

- PAC

- Completion Mammography

- PDOA to be given after MPE report (review of slides) is available.

Ref for PAC

R no ①

Yours



19/5/14

S/Bones
Reviewed in ward

60y ♀

- ~~perimenopausal~~ multiparous

- No comorbidities / Family Hx no malignancy

Cycto-
isegula
Pslr

Q. ① Breast lump ~ 1y

Enlarged at outside. Aged 6 years

lumpectomy on 14/1/14 (PM)

HPE - IDC
(PM) margin unknown
SRL

↓
No Adjunct to second

Q2 - No neck nodes

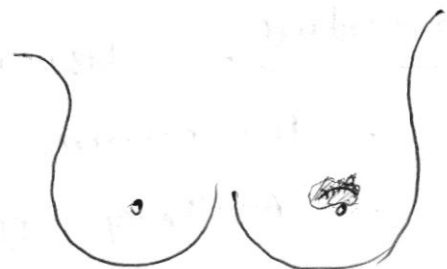
① Breast - 3.2cm lump

Central quadrant

- previous op scar ⊕

- Skin tethering ⊕

- NO axillary LNs



⊕ yc T2 N0 M0

② Breast and axilla : NAB

P/A NAB

Investigations

Slide review

149208

4/2/14

IDC

ER, PR +ve

Her 2mm 3+

- CxL - NAB

- Bk Mamm

1/2/14 (IRM)

② - Normal

① - BIRADS II

- USG Abd

4/2/14 (IRM)

Normal

Δ. ① Cx Breast (T2N0M0)
[post biopsy (outside)
sentinel nodes]

Plan - ① MRM

PA - cleared



28/5/14

surgery - left MRM

findings - seen ⊕ on superior aspect of
nec i ~ 4x4cm palpable lump
- nec, chest wall, skin ⊕
- subcutaneous axillary level 2 in ⊕

procedure - left MRM sent

- per minor, is pedicle, LTV preserved
- Axillary clearance done upto level 2

1
Apix

4/6/14

- Wound - healthy
- Flap well stuck.

• HPR (1417588):- Tumor 2x1x1cm
Margins free
IDC, NOS, Grade II
0/9 L.N

ER +
PR +
Her-2-neu +

Ado - Refers to BCC on Mon/Friday 2pm
for opinion regarding Adjuvant
chemo & hormonal therapy.

See

4/1/18

CSB SR 50

K/d/o CA (L) Breast [T₂ N₀ M₀] (Post lumpectomy
= Unknown Margins) - ER (+), PR (+), HER-2/neu
3+

↓
(L) MRM done at IACH on 28/5/14

↓
WPR-1417588 - T → 2 x 1 x 1 cm
IDC

Grade - 2

Deep Margin - free

o/g

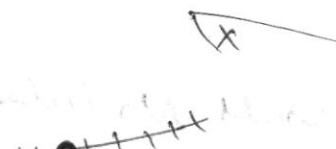
P T₁ c N₀ M₀

↓
Was referred to BCC for Adjuvant Chemo &
Hormonal therapy on 24/6/14

↓
Pt - defaulted for 3 1/2 yrs

↓
Now c/o nodular swelling over chest wall (L)
on scar site

o/g



1.5 x 1 cm hard nodule, mobile
Overlying skin - ? Involved

ChW. Dr. Luman

Adm

1. Core needle biopsy of nodule
2. R/I
3. Metastatic work up

- Review c reports

- To be discussed c Sir.

Kunal Chahal

SR (Co)

5/4/18

Pt. defaulted

CT

IRCM. 28/2/18

Lung - 5mm nodule along oblique
fossa likely benign

4 breast No obvious mass lesion

Imp. No 40 metastasis

Bone scan

MDP-50-12866-18 SP-50
31/1/18

→ No definite 20 skeletal
met

Excision of @ Chest wall Nodule + LA on 15/1/18

HPK

181578
25/1/18 →

Nodule 2.5 x 2 x 1.5 cm
Tumor - 2.5 x 1.8 x 0.6 cm
Invasive Ca

ER+ PR+ Her2Neu 3+
margin free

Ref to Med. oncol for Adj Chemo + Hormonal therapy
opinion

Axilla - clunk @

- L1 8/0 8mm nodes

Kept for Ax. & Surgical oncol.

Flux after ax. and opinion /

Sentinel

05/04/18

Ca Breast (L)

p/m/m

pT1c N0 M0

↓

debulked (no adjacent tissue & HT)

Now local recurrence

Access

(181578)

①

Surgical (Bx) - 8/0
Bx & saw

2.5 x 1.8 cm x 0.6 cm

Invasive Ca NST

BR4 / PR+ / Her2Neu

margin free

②

Bone scan

(31/1/18)

- No eff. skeletal mets

CBCF chest + And + Pelvis (- 28/02/18)

✓ (1) Lungs — ~ 1mm nodul along oblique fissure, Benign

✓ (4) Breast — Not seen

8mm (4) axillary LN +

Abdomen + Pelvis (2)

Imaging

Recurrent (4) Breast (Left)

— left P/ Exclusion of

(left P/ Exclusion of recurrent nodules
(left P/ Pz + / Her 2nd +))

New PS-1

Adv

(1) DIRECTOR

(2) RT-opinions

(3) CBC/BCH

(4) F/U on 13/04/18

~~Adv~~ solution

13/04/16

Ex

Excessional BP (18/15/88)

— left chest wall nodules (2.5 x 2 x 1.5 cm)

left side 2.3 x 1.8 x 0.6 cm

g. Invasive C, NST

BR +ve (8/8)

Ki67 12-14%

PR +ve (8/8)

Her dms 3+ (4/4)

Dr. B.R.A. Institute Rotary Cancer Hospital, AIIMS, New Delhi
**SURGERY PERFORMED AFTER PRESENTATION
AT Dr. B.R.A. IRCH**

margin (2)

— margin free of tumor

— Not affected

for Transmembranes

Plan

① 4 EC → 4 T ± 4

#3 R.P.

②

12 EC

③ F/V av

HT - 14.5 cm

WT - 29 kg

BSD 1.26

Inj. Epirubicin 90 mg/m² ~ 110 mg

Inj. Epirubicin 60 mg/m² ~ 75 mg

AV

8/1/16

9/5/18

Nov 1' 2 & EC

C1 EC - 15/5/18

C2 - 1/5/18

No to picks 80pc

Pls

C # 3 w 15/5/18

- from 18/05/2018 to CRC

✓

Dr. B.R.A. Institute Rotary Cancer Hospital, AIIMS, New Delhi
RADIOTHERAPY

Bhagwati devi
100281097 / 63/F
162206

RT-II - 100360

DOR - 17/4/18

P/w c/o - perimenopausal women
- NO H/O medical/surgical comorbidities

Lump in (L) breast - 2014

↓
Lumpectomy (PVT - 14/1/2014)

PO HPE - Invasive ductal carcinoma
PVT 14/1/14 LVE (L)ve

AIIMS review / 13/2/14 - 144208 - Infiltrating ductal carcinoma
NOS, gr-2, score 7

ER 8/8
PR - 8/8

Her2neu - 3

Mammogram / 1/2/14 - AIIMS / ⇒ Post op change in (L) breast
small intramammary node (benign)

also seen in infero-medial quadrant of (L) breast

(R) breast - BIRADS - 1

USG abd - 4/2/14 / ⇒ (N) study
AIIMS

↓

Surgical oncology
AIIMS - 28/5/14

completion mastectomy

Intraop ⇒ scar (P) on superior aspect of NAC
~4x4cm palpable lump.
NAC, chestwall, skin (N)
Subcm axillary level I LN (P)

Post op HP - 1412588 /
1/6/14 - AIIMS

Tumor - 2x1x1cm
Invasive ductal ca
LN - 0/9

0.8cm - Deep
resected
mass
1cm - overlying
skin

PTcNoMo

No adjuvant treatment received

↓
defaulted for 3 1/2 yrs

Plw Nodular swelling over ① CW
or Scar site

CCT - CHD + P
28/2/18 - 173 M / → 5mm sized nodule - ~~best~~ oblique fissure
likely benign

① breast - not seen, post op bed - no obvious mass seen
8mm sized Subcm axillary node seen

Rest - ② Study

Bone Scan - MDP-50-12866-8
173 MS - 31/1/18 / → NO e/o skeletal mets

↓
underwent excision ① CW nodule
Surgeon's
15/1/18

Post op H/P 11/1/18
25/1/18 15178 / → Tumor - 2.3 x 1.8 x 0.6 cm

Invasive carcinoma, NST

1 x 8/8 11 x 8/8 11 x 2mm 21

67-12.15.18

All margins - free of tumor

↓
planned for Adj. chemotherapy

↓
ABC - 4 T 2 11

1st cc on 15/1/18

Toxicity not
not affordable

Op: F10p TS-2 PT 2 cy 11/1/18

S/P - NAD

Bhagnat devi
100281097 / 62/2
162206

RT-II - 100360
DOR - 17/4/18

perimenopausal women
P/w c/o - NO H/O medical/surgical comorbidities

Lump in (L) breast - 2014

↓
Lumpectomy (PVT - 14/1/2014)

PO HPE - Invasive ductal carcinoma

PV 14/1/14 LVE EYE

ASSMS review /
13/2/14 - 16/2/14

infiltrating ductal carcinoma
NOS, G-2, score 7

FR 8/8
PR - 8/8

Mammogram / > Post op change in (L) breast
1/2/14 - 1/2/14

also seen in inferomedial quadrant of (L) breast

(R) breast - PLEADS - 1

USG abd - 4/2/14 / > (N) Study
ASSMS

Surgical oncology
ASSMS - 28/5/14

Completion mastectomy

Intraop → Superior aspect of NCC
4x4cm palpable lump.
NCC, chest wall, skin (N)
Subcm axillary level LN (N)

Post op HPE - 14/7/14
1/6/14 - ASSMS

Tumor - 2x1x1cm
Invasive ductal carcinoma
IN - 0/9
T, N, M

No adjuvant treatment received

↓
defaulted for 3 1/2 yrs

↓
P/w Nodular swelling over ① CW
on Scar site

CECT-CT+P
28/2/18 - AIMS ⇒ 5mm sized nodule - ~~not~~ oblique fissure
likely benign

① breast - not seen, post op bed - no obvious mass seen
8mm sized Subcm axillary node seen

Rest - (N) study

Bone Scan - MDP-50-12866-18
AIMS - 31/1/18 ⇒ NO e/o skeletal mets

↓
underwent excision ① CW nodule
Surge, onco
15/1/18

postop HP AIMS
25/1/18 181578 ⇒ Tumor - 2.3 x 1.8 x 0.6 cm
Invasive carcinoma, NST
ER 8/8 PR 8/8 Her2neu 3+

Ki67 - 12-15%
All margins free of tumor

↓
planned for Adj. chemotherapy

4EC - 4 T ± 4

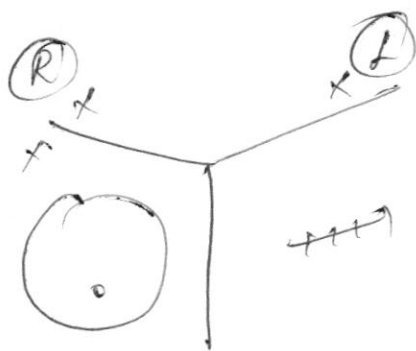
Trastuzumab
not affordable

↓
1st EC on 15/4/18

O/E: ELOB PS-2 P-I-ly-CR-LAD-EB

S/E - NAD

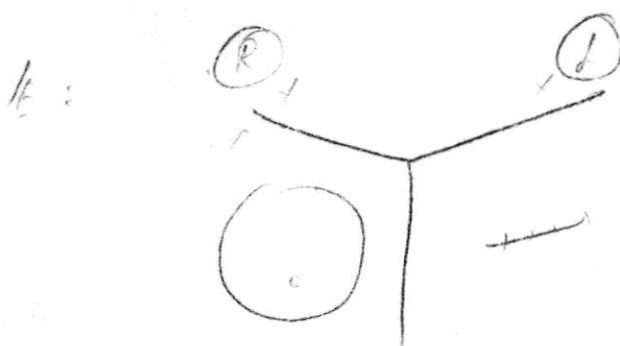
L/E =



Recurrent Ca (L) breast ER 8/8
 (2014) post MRM PR 8/8
 post excision Her 2neu 3+

Plan: - To continue chemo (↓ MD) (4# EC → 4# D)
 ↓
 #b.
 - Local RT to (L) CW ± electrons < 50 Gy / 25# / 5 weeks >.
 #b
 HT

Pitishu



Recurrent (a) ① breast TR 8/8
 post excision PR 8/8
 Her2neu 3+

plan: